

340B Reporting Guidance – FY25

Narrative Supplement

(1)(A)(ii)(III) and (IV) Payment of Administered Drugs

All payment amounts for administered 340B drugs were reported as bundled payments due to limitations in our ability to reliably identify drugs reimbursed separately from those reimbursed as part of an encounter within our system.

The wholesale acquisition cost (WAC) comparison methodology described in the instructions does not appear to correspond to how payments are determined or received by the hospital, as payer reimbursement is not based on WAC and does not vary based on 340B eligibility. Accordingly, the hospital estimated payment amounts using a proportional allocation derived from actual encounter reimbursement ratios. The calculation is complex and the reported amounts represent good faith estimates intended to approximate payments received.

We would like to reiterate the concern that we shared with the GMCB Prescription Drug Affordability Team on January 26, 2026, regarding the reporting requirements in this section of aggregated payment amounts for administered drugs paid separately and administered drugs paid by bundled payments. Because payment amounts are the same for all third-party payors regardless of 340b eligibility, reporting this data as described does not accurately reflect the value of the 340B program at Springfield Hospital.

All payors are billed for the same amount for the same medications, typically within three business days of service. Weeks to months later, if appropriate 340B eligibility criteria are met, eligible encounters are identified and the hospital may replenish its accumulated medication supply at a lower 340B price. Regardless of whether a drug is later deemed 340B eligible, the payer is always charged the same amount.

Therefore, the value of the 340B program to the hospital is the difference between the GPO acquisition price and the 340B acquisition price. Reporting overall aggregated payment amounts for administered drugs would significantly overestimate the financial impact of the 340B program.

For example:

Drug Charge: $200
Payment received (80% payment from BCBS): $160
GPO cost of drug: $100
340b cost of drug: $80

Under this reporting approach, the 340B margin may be perceived as $80 (160-80), when the true value of the 340B program is $20 ($100-$80).

We respectfully request that this be taken into consideration when reporting on "aggregated payment amounts" as this approach would misrepresent the value of the program.

(1)(B)(4) A description of the ways in which the hospital uses revenue from its participation in the 340B program to benefit its community through programs and services funded in whole or in part by revenue from the 340B program, including services that support community access to care that the hospital could not continue without this revenue.

As a Critical Access Hospital serving a rural community, participation in the 340B Drug Pricing Program is essential to sustaining local access to care. Our hospital provides a high level of charity care and experiences significant bad debt due to the financial challenges faced by our patient population. In addition, Medicaid reimbursement consistently falls short of the actual cost of providing care, creating an ongoing Medicaid shortfall. As a small rural hospital, it is also challenging to absorb the gap between provider tax assessments and Disproportionate Share Hospital (DSH) payments.

Savings generated through the 340B program help offset uncompensated care costs and reimbursement gaps, while also helping to absorb the expense of high-cost medications that do not qualify for 340B pricing. The savings help us to maintain 24/7 emergency services and support highly utilized outpatient and specialty services. Without the benefits of the 340B program, it would be significantly more difficult to continue offering certain essential

services locally, negatively affecting access to care in our rural community. Ultimately, the 340B program is a critical component of our hospital's financial sustainability and our ability to serve the community.

(1)(B)(5) A description of the hospital's internal review and oversight of its participation in the 340B program in compliance with the U.S. Department of Health and Human Services, Health Resources and Services Administration's 340B program rules and guidance.

From SH 340B Program Policy, Procedure Section:

340B Program Oversight

1. The oversight of the 340B Program is the responsibility of the 340B Program Committee which comprises the following:

1.1. Authorizing Official

1.2. Primary Contact

1.3. CEO

1.4. Billing, TMS, etc. as needed

2. The 340B Oversight Team has the following responsibilities:

2.1. Setting the general direction and policy for 340B drug purchasing and compliance. 6

2.2. Establishing a 340B Program audit plan to:

2.2.1. Ensure that internal and external self-audits are conducted on a regular basis in accordance with 340B Program auditing as set forth by SH's Oversight Team.

2.3. Meetings occur annually to review financial and 340B related reports, trends, audit results, and topics.

2.4. Staying abreast of information on current best practices by sending key personnel to related conferences and/or training programs (e.g., webinars and/or teleconferences).

2.5. Providing compliance and oversight direction.

2.6. Providing appropriate resources for 340B Program oversight.

2.7. Determining 340B Program modifications or expansion.

2.8. Communicating to SH's leadership of potential changes to the 340B Program that could impact the 340B Program.

2.9. Assessing discrepancy response, including the determination of an occurrence in which a material breach has been met.

A description of how the aggregated payment amount is calculated for payments made for all other internal expenses related to administering the 340B program during fiscal year 2025.

The internal expenses reported represent the cost of employing .50 FTE dedicated to administrative 340B program duties. This includes salary and associated fringe benefits allocated to 340B program administration.